# Workshop pack: a 90-minute test of one delayed-transfer decision

*Facilitator pack. Draft, 1 November 2026. Prepared by [Your name]. Synthetic scenario only: no client data, no patient details, no contact with the HSE.*

## Purpose and rules

The workshop decides one thing: stop, assess a real workflow, or design a shadow-mode proposal. It must end with one candidate engagement or a stop. The decision is non-clinical. Patient selection, clinical review and discharge decisions are out of scope.

**Before the session:** confirm attendees [names to confirm] (Life Sciences, AI & Data or digital twin, health operations, risk and independence), and clear the independence check. Allow about two hours of preparation.

## Agenda

| Time | Segment | Output |
|---|---|---|
| 0-10 min | Purpose, boundaries, stop conditions | Agreed scope |
| 10-25 min | The synthetic scenario | Shared picture of the decision |
| 25-40 min | Map presumed current practice: who decides, what is recorded, where | Owners and records, marked as assumptions |
| 40-60 min | Complete the decision contract (template below) | Draft contract |
| 60-75 min | Red-team: duplication, capacity, burden | Evidence for or against the gap |
| 75-85 min | Individual scoring | Completed feedback forms |
| 85-90 min | Decide | Stop, assess a real workflow, or design a shadow-mode proposal |

## Synthetic scenario (not real data)

A hypothetical acute hospital's midnight count of delayed transfers is over its site threshold for the third day running. Eight patients are recorded as waiting for residential care. Four have an approved funding decision but no place has been offered. Two are waiting for a funding decision. One family has declined an offered place. One placement offer has been outstanding for six days.

The question for the group: when the threshold is breached, who decides what, by when, and how is the result recorded? Which of the eight are a missing bed, and which are a decision waiting?

## Blank decision contract

| Field | Participant input |
|---|---|
| Decision to be governed | |
| Scope and exclusions | |
| Trigger and evidence source | |
| Accountable owner and deputy | |
| Acknowledgement deadline | |
| Decision or escalation deadline | |
| Permitted non-clinical actions | |
| Escalation route | |
| Reason to be recorded | |
| Feasibility class (decision delayed / action infeasible / action done without expected result) | |
| Expected outcome and review point | |
| Existing systems and records that may already hold this | |
| What would disprove the need? | |

## Five facilitation questions

1. Does a real decision like this exist in any EY engagement or client conversation we know of?
2. Does the current process already identify an owner, deadline, reason, escalation and outcome reliably?
3. Could an existing tool or process provide this record through ordinary configuration?
4. Would unavailable capacity, rather than a waiting decision, explain most of the eight cases?
5. Would this contract reduce ambiguity, or just add another administrative step?

## Feedback form

1. Is the decision specific enough to assess? Yes / No / Unclear
2. Is there a plausible gap worth testing? Yes / No / Unclear
3. Is duplication of existing tools or processes likely? Low / Medium / High
4. Would this add useful control without excessive burden? Yes / No / Unclear
5. Decision: **Stop / Assess a real workflow / Design a shadow-mode proposal.** Reason: _____

## After the session

Record the decision and the candidate engagement (or the reason to stop) within one working day. If the outcome is Assess, Step 2 needs a named sponsor, a client route, resource approval and IP and independence clearance before week 1.